INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Vinorelbine capsules · advanced breast cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00259
Version 6

Once-weekly capsules

Treatment stages

01 **Vinorelbine capsules · advanced breast cancer**
7-day reference

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Aim to slow or shrink advanced breast cancer and help symptoms while balancing side effects. Duration depends on benefit, tolerability and the agreed plan; a fixed number of cycles or a guaranteed survival improvement is not promised.

VINCA ALKALOID CHEMOTHERAPY

Vinorelbine

Disrupts the machinery of cell division

Binds cell-division structures → Microtubules cannot function normally → Cell division is disrupted

Vinorelbine disrupts the tiny structures a cell needs to divide. This can stop cancer cells multiplying. Healthy blood-forming cells, gut cells and nerves can also be affected.

Use the treating unit's 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

The treatment being discussed

Breast treatment context

Advanced breast cancer. This patient suite describes the breast setting; the source protocol also has other tumour indications with their own clinical context.

NCCP 00259 v6 · official regimen

Schedule and duration

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4,80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity.60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

NCCP 00259 v6 · official regimen

How treatment is given

Take with some food; swallow capsules whole with water. Do not chew, suck or dissolve. The exact total dose may require more than one capsule; pharmacy provides the strengths and number. Verify the weekly date, dose and capsule count each time. Do not take another dose if vomiting occurs within a few hours. Contact the team and record the dose taken. Do not swallow damaged capsules. Return them to pharmacy; immediately wash any contact with contents using water or saline. Missed/late dose: contact the team for the next date. Do not double or move the weekly schedule yourself. This is pragmatic medication-safety advice, not an extra NCCP catch-up schedule.

NCCP 00259 v6 · official regimen

Why this treatment may be chosen

The oral weekly schedule has shown activity in advanced breast cancer. It needs blood tests and toxicity review even though capsules are taken at home; oral and intravenous (IV) doses cannot be swapped.

Evidence and alternatives are discussed for your exact setting. Trial survival averages are not an individual prediction.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Use the treating unit's 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones are not equal time intervals. Your team books and reviews tests. Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00259 v6 · official regimen · Current Irish product information · safety supplement

THE WHOLE TREATMENT JOURNEY

Vinorelbine capsules · advanced breast cancer: treatment and checks

Vinorelbine capsules · advanced breast cancer

7-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

Advanced breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00259	Confirm breast plan and current dose	Weekly doses 1–3; review before dose 4	Continue while helpful and tolerated	Review dose and recovery before restart	Agree continuation / next treatment
Blood / kidney / liver NCCP 00259 · SMPC	Baseline blood and organ tests	Counts before every weekly dose	Weekly treatment review	Review recovery and current dose	Follow unresolved abnormalities
Symptoms / medicines NCCP 00259 · SMPC	Bowel / nerve / infection review	Report early symptoms and support problems	Review all medicines and side effects	Do not restart or escalate yourself	Agree continuation and response assessment
Special schedule check NCCP 00259 · SMPC	Confirm weekly capsule plan	Review nadirs after doses 1–3	Dose increases only after team review	Keep previous dose / nadir history	Personal plan; no booked dates implied

What each check is for

1



Treatment and dose review

Before every administration

Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

Ask / act: The treating team confirms results, dose and suitability; this map does not authorise treatment.

2 **Support and symptoms**

Throughout treatment, including days at home

Use the treating unit's 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

Ask / act: Keep a written medicine plan and urgent-contact route.

3 **Response and continuation**

At agreed clinical review points

Aim to slow or shrink advanced breast cancer and help symptoms while balancing side effects. Duration depends on benefit, tolerability and the agreed plan; a fixed number of cycles or a guaranteed survival improvement is not promised.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks before and during treatment

Every dose depends on the reviewed treatment plan and results.



Blood and symptom checks

NCCP 00259 v6 · official regimen

Heart and response checks

Review cardiovascular history and new symptoms. These monotherapy NCCP protocols do not mandate the serial left ventricular ejection fraction (heart pumping measurement) (LVEF)/echo schedule used for anthracyclines or trastuzumab; arrange cardiac tests when clinically indicated.

NCCP 00259 v6 · official regimen

Fertility, pregnancy and breastfeeding

Purpose and alternatives for the person’s advanced breast cancer, including other systemic treatment or supportive care according to goals. Possible benefit is uncertain; response is reviewed and treatment can be paused, changed or stopped. Low blood counts, infection/sepsis, fatigue, nausea/vomiting, bowel problems, neuropathy, hair change and uncommon serious lung/heart/allergic toxicity. Fertility and pregnancy planning before treatment; discuss sperm/egg preservation where relevant. Do not breastfeed. HPRA supplements: contraception during treatment and for 7 months afterwards for women,4 months for men; confirm the exact product and individual advice.

NCCP 00259 v6 · official regimen

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Vinorelbine capsules · advanced breast cancer

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Weekly capsules · Day 1	 **Day 1** Only after team clearance
Blood counts / infection	 **Days 1–7** Fever or sudden illness needs urgent help on any day
Nausea / bowel symptoms	 **Days 1–7** Use the prescribed support plan
Nerve / fatigue symptoms	 **Days 1–7** Report new or worsening symptoms early

Treatment and recovery: ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4,80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity.60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

Tests and decisions: Blood counts are checked on the day of each weekly dose. Kidney and liver tests follow the weekly regimen and prescribed plan. The team reviews the lowest counts after earlier doses before any increase, alongside bowel, nerve, infection and medicine checks.

> **Why these bands stay broad:** This is a relative reference, not an appointment calendar. Treatment days can change after clinical review. Symptom bands do not predict when an effect will occur. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00259 v6 · official regimen ↗

> **There are no guaranteed "safe days".** A day without treatment is not automatically a safe day. Use the emergency contact when needed; do not move or restart doses yourself.

Support and safety at home

Keep the written medicine and urgent-contact plan.

Sickness plan

Take the anti-sickness medicine exactly as prescribed before/around the weekly capsule dose; contact the team if it is not controlling symptoms.

NCCP 00259 v6 · official regimen

Bowel care

Agree a constipation plan, particularly if using opioids or ondansetron. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment; do not simply add laxatives.

NCCP 00259 v6 · official regimen

Medicines and vaccines

Check every new prescription, supplement and vaccine with the team; avoid grapefruit. Live vaccines and interacting medicines need review.

NCCP 00259 v6 · official regimen

Weekly safety

Take capsules only once weekly after the team clears the blood results. Do not repeat a vomited dose or increase the capsules yourself.

NCCP 00259 v6 · official regimen

Urgent contact

NCCP 00259 v6 · official regimen

Practical support

Sickness plan

Bowel care

Medicines and vaccines

Weekly safety

Taking tablets / attending treatment

Practical treatment details

Treatment preparation

Record actual dose, capsule strengths/count, date taken, vomiting and side effects in the passport.

At home

Ensure access to a refrigerator if required by the dispensed pack; discuss travel/transport with pharmacy.

Questions to ask

Can IV and oral vinorelbine be exchanged?

Oral 60/80 mg/m² and IV 25/30 mg/m² have pharmacokinetic equivalence references, but never convertmg-for-mg or swap routes yourself. Dose rules and schedules differ.

How do I store the capsules?

Current Vinorelbine Accord SmPC: refrigerator 2–8°C. Confirm the dispensed brand/pack instructions; keep in original packaging and out of children's reach. Return unused or damaged capsules to pharmacy; follow cytotoxic handling advice.

Does dose four always use more capsules?

No. The team reviews the lowest counts and side effects after the first three doses. Take only the current prescribed dose; severe or repeated low counts can prevent an increase.

Explain the plan back: Which treatment days and doses are prescribed, which blood tests must happen first, how do you take the support medicines, and which symptoms need urgent help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Vinorelbine capsules · reviewed support plan

Active medicines: Vinorelbine

Emetogenic risk: Moderate-to-high oral risk

Medicine	Dose / route	When
One 5-HT3 antagonist · choose a single option	Ondansetron 8–16 mg total daily OR granisetron 1–2 mg total daily	Around the weekly dose; the prescriber must state the number of subsequent days NCCP Table 5 oral pathway. Do not combine both agents or assume indefinite daily treatment. Review constipation, QT risk and interacting medicines.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibacterial, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by these NCCP monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.

- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.
- Confirm strong CYP3 A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- Table 5 says continue daily; for once-weekly vinorelbine the prescriber must state the number of days. Do not automatically turn this into indefinite daily treatment or import an intravenous (IV) moderate steroid/NK 1 schedule.
- A separate locally approved as-needed plan; check constipation and QT/interactions. Avoid duplicate 5-HT3 agents. Ondansetron can worsen constipation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Vinorelbine capsules · advanced breast cancer

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4,80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity.60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

Oral chemotherapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Blood counts and infection

Vinorelbine

White blood cells can fall, raising the risk of infection. Anaemia can cause tiredness or breathlessness; low platelets can cause bruising or bleeding. Blood tests guide delays and dose changes. Fever, shaking chills or feeling suddenly unwell need the oncology emergency contact immediately.

3 Tingling, numbness or weakness

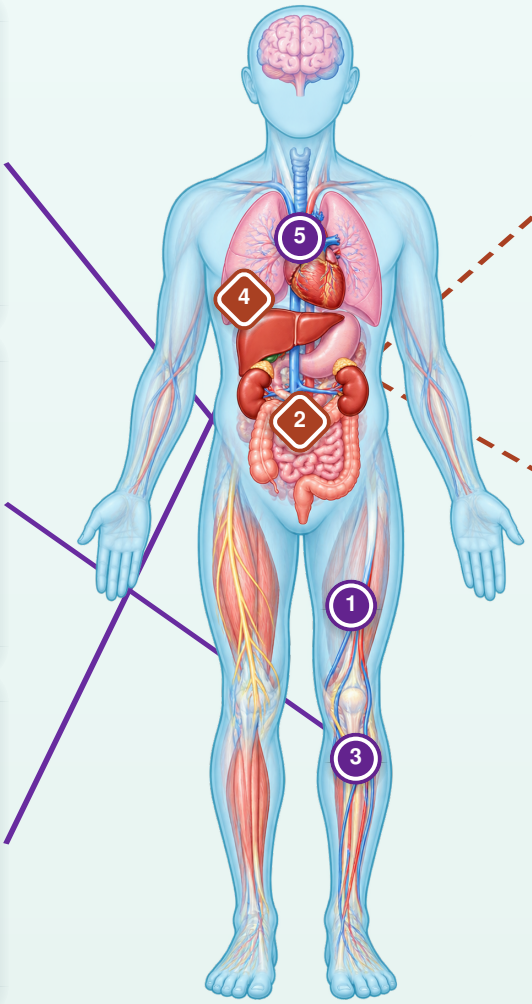
Vinorelbine

Nerve effects may change sensation, grip, walking or bowel movement. Report changes early because the next dose may need to wait or be reduced.

5 Uncommon serious symptoms

Vinorelbine

New chest pain, sudden or worsening breathlessness, or severe allergic symptoms need urgent medical help. Previous heart disease and interacting medicines need review before treatment.



2 Constipation, nausea and mouth problems

Vinorelbine

The bowel can slow down and nausea, vomiting, diarrhoea or sore mouth can occur. Use the prescribed bowel and anti-sickness plan. Severe abdominal pain, swelling, repeated vomiting or inability to pass stool/gas needs urgent assessment for ileus; do not simply take extra laxatives.

4 Fatigue, hair and liver tests

Vinorelbine

Tiredness, hair thinning/loss and liver-test changes can occur. Pace activities and tell the team if eating, drinking or daily tasks become difficult.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use the treating unit's 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Fatigue	☐	☐	☐	
Nausea	☐	☐	☐	
Bleeding / bruising	☐	☐	☐	
Numbness / tingling	☐	☐	☐	
Constipation	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Use the treating unit's 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit. Emergency symptoms: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Vinorelbine capsules · advanced breast cancer

NCCP 00259 v6

ONCE WEEKLY. First 3 administrations 60 mg/m², total maximum 120 mg. From administration 4,80 mg/m² (maximum 160 mg) only after the team reviews nadirs and toxicity.60 or 50 mg/m²/lower may be needed. Continue weekly while helpful and tolerated; no fixed total number of weeks.

Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Use the treating unit’s 24-hour emergency number and local fever threshold. Contact immediately for fever/chills, sudden illness, breathlessness/chest pain, uncontrolled vomiting, severe abdominal pain/obstruction symptoms, bleeding or severe allergy. Do not wait until the next visit.

https://sactcheck.com/patient/00259/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.20 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00259 v6 · official regimen ↗**
   https://healthservice.hse.ie/documents/6363/259_Vinorelbine_Oral.pdf
2. **Current Irish product information · safety supplement**
   https://assets.hpra.ie/products/Human/38141/Licence_PA2315-253-001_09042025122220.pdf
3. **NCCP antiemetic guidance v6 · oral Table 5**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCCP NCIS antiemetic medicines v8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **Freyer G et al. Journal of Clinical Oncology. 2003;21:35–40. DOI 10.1200/JCO.2003.09.057**
   https://pubmed.ncbi.nlm.nih.gov/12506167/